

Our Ref: (T) TS/AS/ [MED-ID]

[PROVIDER-NAME]

[PROVIDER-NAME]

[PROVIDER-NAME]

[PROVIDER-NAME]

Joint IBD Clinic

[DATE]

Clinic Letter [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Division B

Care Group - Emergency & Nurse [NAME] Medicine Gastroenterology

Room CG89, Mailpoint [ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

[PHONE] -->

Dear [PROVIDER-NAME] [NAME] , 48 Highlands, Potterne, Devizes, Wiltshire. [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

Diagnoses:

Active Crohn's disease with previous right hemicolectomy

Treated with high-dose [NAME] (10 mg/kg)

Treated with subcut Methotrexate

Recent abnormal LFT

Worsening symptoms following recent withdrawal of steroid therapy

I was asked to see Mrs [NAME] urgent extra in my clinic this morning. [NAME] is well known to colleague [PROVIDER-NAME] and we discussed her case over the telephone.

Essentially since stopping her Prednisolone 10 days ago [NAME] has had worsening right iliac and this is very evident when I examined her this morning. [NAME] does not describe any obstructive symptoms and [NAME] is no fever. [NAME] are no physical signs to suggest either an abscess or perforation.

We have concluded that the [NAME] is not working. Additionally her LFTs have deteriorated on Methotrexate and I have advised that we now stop this.

We are going to switch the [NAME] to Ustekinumab. [NAME] will be seen this morning by Maz Bet Nurse [NAME] nurse who oversees the biologic service and we will aim to get her up for an Ust infusion as soon as possible.

We will see her back in the clinic in six weeks' time.

With best wishes

Yours sincerely

[NAME]

[NAME]

[DICTATION-META]

[PROVIDER-NAME]

Consultant Gastroenterologist

Copies to:

[PROVIDER-NAME]

[PROVIDER-NAME]

Sister [NAME] [NAME]